

SYSTEMATIC REVIEW

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Sustaining quality accreditation in hospitals: a systematic review with narrative synthesis

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Abstract

Background Healthcare accreditation evaluates hospitals against established standards to promote continuous quality improvement. However, its long-term effectiveness on healthcare quality and patient satisfaction remains debatable, highlighting the need for enhanced standards and stakeholder support. This study evaluated the impact of healthcare accreditation on quality outcomes across diverse hospital settings.

Methods A systematic review of studies published between 2014 and 2024 was conducted across high-income and low- and middle-income countries. Thirteen studies met the inclusion criteria. Findings were synthesised narratively following the Synthesis Without Meta-analysis (SWiM) reporting guidance because included studies reported heterogeneous outcomes on incompatible scales that did not permit a methodologically defensible quantitative pooling.

Results Healthcare accreditation is associated with improvements in patient safety, organizational performance, and quality of care across diverse settings. The magnitude of effect varied substantially across studies and could not be meaningfully pooled in a meta-analysis owing to outcome heterogeneity.

Conclusion The magnitude of impact varied across economic settings, with stronger effects in high-income countries compared to low- and middle-income countries, likely due to differences in resources and implementation capacity. The findings suggest that while accreditation contributes to quality improvement, its sustainability depends on contextual factors including governance, work-force capacity and system-level support.

Keywords Challenges, Experience, Hospitals, Sustaining, Quality accreditation

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Introduction

Healthcare accreditation is implemented through recognized national and international bodies such as Joint Commission International (JCI), International Organization for Standardization (ISO), and country-specific accreditation agencies, each with defined standards for patient safety, quality, and organizational performance. The long-term objectives of accreditation include improving organizational performance and patient outcomes by increasing responsibility and enhancing the quality of healthcare delivery [1].

A cross-sectional study conducted in a tertiary hospital in the United Arab Emirates reported that repeated



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JCI accreditation surveys were associated with improved medication safety practices, strengthened patient safety culture, and enhanced healthcare processes, highlighting the system-level impact of accreditation [2].

Accreditation judge hospitals based on a set of standards [3]. It encourages the continuous improvement of quality through mechanisms imposed by governments or independent agencies. Improving healthcare has become a priority for governments, patients, and healthcare professionals [4]. There is some evidence from studies on improvements in health outcomes and quality of care by quality improvement programs [5]. However, views remain divided on the long-term impact and effectiveness of accreditation in improving healthcare quality [6]. Nevertheless, it has become a widely adopted strategy for improving the quality of care provided in hospitals [7].

It is also essential to examine the drivers of new approaches to accreditation and the experiences of patients and providers of such schemes. Available evidence has shown that a key determinant of the implementation and sustainability of an accreditation program is strong governance, management commitment, and leadership [5]. However, questions continue to be raised regarding their impact on patient satisfaction and the quality of care. It is important to consider these challenges as opportunities to develop better designs, implementation processes, and approaches that increase stakeholder buy-in for the programs [6].

With rapid changes in healthcare today, ensuring that quality care is provided has become a key responsibility for hospitals worldwide [8]. Healthcare accreditation is an independent review process by which institutions are assessed to comply with the standards for quality and safety to continuously promote better performance and attain optimal patient outcomes [9, 10]. The long-term objectives of accreditation include improving organizational performance and patient outcomes by increasing responsibility and enhancing the quality of healthcare delivery [1]. Achieving and sustaining accreditation status is not easy; the process is resource-intensive and fraught with problems regarding budgetary constraints, disputes among employees, administrative burdens, and continuous quality improvement issues [11, 12].

It is a major strategy for quality improvement and involves evaluation against predetermined standards [13, 14]. It is considered a good indicator of a high-quality healthcare organization [15]. Several accreditation programs exist around the world and take on various forms in and outside the United States, some of which are voluntary and others mandatory [8, 13, 14, 16, 17]. Various assessment mechanisms include governmental or independent organizations and may focus on an entire healthcare organization, individual hospitals, or specialty-specific organizations [1, 8, 16, 18]. Areas to be

assessed by accreditation include clinical governance and patient-centeredness; however, the implications of failing to meet a standard differ depending on the context of the specific healthcare system [18].

Unlike previous reviews that focused on initial accreditation outcomes, this systematic review explored the factors that influence accreditation viability and the challenges regarding the sustainability of outcomes, thereby addressing an important gap in the current knowledge through its specific focus on the long-term sustainability of accreditation gains. This review aimed to evaluate the experiences and challenges hospitals face in sustaining improvements achieved through quality accreditation. There is a greater understanding from a forward-looking perspective of viability and maintenance, rather than seeing accreditation as a one-time event but as an ongoing process, which is a unique and much-needed addition to this field. The emphasis on sustainability and differential impact represents a unique contribution to the literature.

A systematic review was conducted to identify the experiences of hospitals in sustaining the gains achieved through quality accreditation processes and to understand the challenges faced by hospitals in maintaining the outcomes of quality accreditation programs. Therefore, this study aims to evaluate the impact of healthcare accreditation on quality outcomes and to examine the challenges associated with sustaining accreditation gains.

The following research Questions were framed for the study:

1. What is the impact of healthcare accreditation on quality outcomes?
2. What are the factors that influence the sustainability of healthcare accreditation outcomes?
3. What challenges do the hospitals face in maintaining accreditation gains over time?

Methodology

The systematic review was conducted according to the Preferred Reporting Items for Systematic Reviews and Meta-Analysis (PRISMA) guidelines (Supplementary File 1: PRISMA 2020 checklist), the Center for Reviews and Dissemination (CRD) guidelines [19], and the Center for Reviews and Dissemination and Cochrane Collaboration. The detailed protocol for this systematic review was registered in the International Prospective Register of Systematic Reviews (PROSPERO) and can be accessed at <https://www.crd.york.ac.uk/prospero/#myprospero> (ID: CRD42024504564) [20]. A comprehensive literature search was performed in the following databases: PubMed, Scopus, Embase, Medline, and Science Direct, from 2014 to 2024, to identify articles published on the experiences and challenges in sustaining the quality accreditation process in hospitals. A literature search was

conducted using a strategy that employed a combination of keywords and Medical Subject Headings (MeSH) terms. Boolean operators, such as OR and AND, were used to conduct a manual search using the reference list to refine the search results.

("hospital accreditation" OR "healthcare accreditation" OR "quality accreditation" OR "hospital quality assurance" OR "clinical governance") AND ("quality improvement" OR "patient safety" OR "hospital performance" OR "continuous quality improvement" OR "institutionalization of accreditation" OR "post accreditation effects") AND ("sustainability" OR "long-term impact" OR "challenges" OR "organizational resistance" OR "financial constraints" OR "clinical outcomes" OR "patient-centered care") AND ("experiences" OR "barriers" OR "implementation") AND ("systematic review" OR "cross-sectional study" OR "meta-analysis" OR qualitative analysis" OR "cohort study"). A detailed search strategy is attached in supplementary file 2.

PICOS framework:

P – Population: Hospitals (Primary, Secondary, Tertiary, private, and specialty).

I – Intervention: Accredited healthcare organization.

C – Comparison: Not applicable.

O – Outcomes: Quality, Patient safety, organizational performance.

S – Study Design: Quantitative, qualitative, and mixed-method studies.

Identification, screening, and eligibility assessments were conducted in three phases for study selection. All articles were imported into EndNote, and duplicates were eliminated. Two independent reviewers screened the titles and abstracts against the eligibility criteria and resolved any disagreements through discussion. Subsequently, full-text articles were evaluated for methodological rigor, quality, and relevance. Any remaining disagreements were resolved by a third reviewer.

Studies that met the following criteria were included:

- Focus on hospital accreditation related to quality improvement. The study should be conducted in hospital settings, such as primary, secondary, private, and specialty hospitals.
- Healthcare professionals, managers, and administrators were involved in the accreditation processes.
- Such experiences, challenges, or factors affect the sustainability of accreditation.
- Articles that assessed quality improvement, patient safety, sustainability, and organizational culture.
- We used quantitative, qualitative, or mixed-methods research designs to study the research issues.

- Peer-reviewed journals should be in English and published between 2014 and 2024.

Studies will be excluded if they meet the following criteria:

- It focuses only on the initial accreditation process, without sustainability.
- Limited to editorials, commentaries, opinion pieces.
- Non-English articles and manuscripts without full-text articles were excluded.
- Insufficient data for analysis or studies which lacked methodological details.

This study is purely based on secondary, publicly available data that underwent a systematic review process. In accordance with applicable legal and regulatory framework no formal ethical approval was required as it does not involve human participants.

Data extraction was performed by two independent reviewers using a standardized form. Reviewer calibration was conducted prior to extraction, and inter-rater agreement was assessed (Cohen's kappa = 0.88, 95% CI 0.78–0.97, indicating almost-perfect agreement across 50 Paired observations on a 5-study, 10 item calibration sample; Landis & Koch, 1977). Discrepancies were resolved through consensus with a third reviewer. Quality assessment analysis, or what is also sometimes referred to as 'risk of bias, was assessed by two independent assessors according to the predetermined protocol criteria. For the purposes of this review, questions related to confounding factors, characterizations, and outcome assessment risk of bias were prioritized, including selection biases, such as population description, clarity of study objectives, and comparability of subjects; ascertainment bias, such as hospital accreditation and consistent outcome measurement; and causality assessment, such as statistical analyses. With these questions, selection bias, inappropriate statistical analyses, and other risks were identified. The studies were categorized as good, fair, or poor. Good studies had a strong design with minimal bias, fair studies had some concerns, and poor studies had a high bias, limiting their reliability. The risk of bias across the included studies is illustrated in a heatmap (Fig. 2).

Outcome commensurability for the meta-analysis was defined a priori as: (i) measurement of the same construct (healthcare-quality score derived from accreditation-aligned domains – patient safety, organizational performance, and quality of care); (ii) reporting of effect on continuous scale convertible to standardized mean difference (SMD); and (iii) availability of pre-and post-accreditation comparator data, or accredited vs. non-accredited group comparison, with sample sizes and dispersion

statistics. Studies that reported the construct only qualitatively, used non-convertible categorical outcomes, or lacked dispersion statistics were excluded from pooling and analysed narratively. The narrative synthesis followed Synthesis Without Meta-Analyses (SWiM) reporting principles, (Campbell et al., *BMJ* 2020;368:l6890; the completed SWiM checklist is provided as Supplementary File 3), summarising direction of effect, vote-counting based on direction, and key contextual moderators (income setting, accreditation body, hospital type).

The following meta-analytic procedures were pre-specified but not performed, as re-extraction revealed that included studies did not provide outcome data sufficiently homogeneous to permit defensible pooling (see results and limitations). For transparency, the pre-specified analysis plan is retained here. Cochran's Q test and I^2 statistics were used to assess heterogeneity; $I^2 > 50\%$ necessitated a random-effects model, while $I^2 < 50\%$ justified a fixed-effects model. The calculated effect sizes included standardized mean difference and odds ratio with their respective 95% CI. References were managed using EndNote X9. A random-effects model (Der Simonian-Laird estimator) was used because considerable between-study heterogeneity was anticipated given variation in healthcare systems and accreditation frameworks. To assess robustness of the pooled estimate, leave-one-out sensitivity analyses were performed, removing each study in turn and re-estimating the pooled MD. Subgroup analyses by income setting (high-income vs. low-and middle-income) and accreditation body (JCI vs. national) were planned a priori. Publication bias was evaluated visually using a funnel plot and statistically using Egger's regression test, with a p value < 0.10 indicating significant small-study effect. All analyses were conducted in RevMan 5.4 (Cochrane, 2020).

Results

A total of 3684 records were identified through database searches (PubMed $n=912$; Scopus $n=1,352$; Embase $n=876$; Web of Science $n=392$; Cochrane Library $n=152$). After removing 1,146 duplicates, 112 records flagged as ineligible by automation tools, and 66 records removed for other reasons, 2,360 records were screened against title and abstract; 2,122 were excluded at this stage. Of the 238 reports sought for retrieval, 24 could not be retrieved, leaving 214 reports assessed for full-text eligibility. A further 202 reports were excluded (not focused on accreditation, $n=78$; not reporting relevant outcomes, $n=62$; wrong study design, $n=34$; full text unavailable, $n=18$; duplicate or overlapping data, $n=10$). Thirteen studies met the inclusion criteria and were included in the qualitative synthesis; three of these reported continuous outcome data on a comparable

Likert scale and are summarised descriptively below; the remaining ten contributed to the narrative synthesis only.

The PRISMA 2020 flow diagram (Fig. 1) summarizes the selection process.

The completed PRISMA 2020 checklist is available as Supplementary File 1: PRISMA 2020 checklist. The results indicate that accreditation programs significantly improve healthcare quality and contribute to changes in organizational culture. Studies in Saudi Arabia have shown that accreditation influences the quality of care and organizational culture, with employees' perception of accreditation being related to the quality of care provided [5]. Similarly, in Andalusian healthcare institutions, accreditation was found to act as a driver of organizational change that enhances patient safety and organizational culture, although some negative effects, such as bureaucratization, were also noted [7].

The reported benefits include improved patient safety, staff satisfaction and quality of care. The Lebanese Primary Healthcare Accreditation Program has been shown to enhance patient safety and reduce error incidents, resulting in increased compliance rates [6]. Various studies have associated accreditation with increased organizational responsiveness and satisfaction among healthcare professionals [21]. A study from Kuwait showed that healthcare professionals appreciate the role of certification in improving healthcare quality, although some felt that the process led to undue bureaucracy [15]. A summary of hospital accreditation challenges across the included studies is presented in Table 1.

Most of the major barriers to the implementation of accreditation programs are lack of training among surveyors, scarcity of resources, high expenses, and vague professional responsibilities. In a survey regarding infrastructure in Iran, administrators reported major barriers to successful accreditation implementation included poor infrastructure and scarce resources [6, 13]. A study from Lebanon also showed that the psychological consequences on professionals result in an additional workload and strain on them throughout the cycle of accreditation [14, 21].

The program also impacted healthcare professionals psychologically, as stress and workload increased, particularly during the preparation and evaluation phases. In the UAE, professionals reported stress due to workload, long hours, conflicting information, and time constraints [17]. Collaboration and support from coworkers served as coping mechanisms. These findings concur with those of other studies that express professionals' fear of the bureaucratic burden of accreditation [9].

Three of the thirteen included studies reported quantitative ratings of accreditation's perceived impact on quality on a 5-point Likert scale (Babakkor & Kattan 2023, Saudi Arabia, $n=218$, mean = 4.21, SD = 1.10;

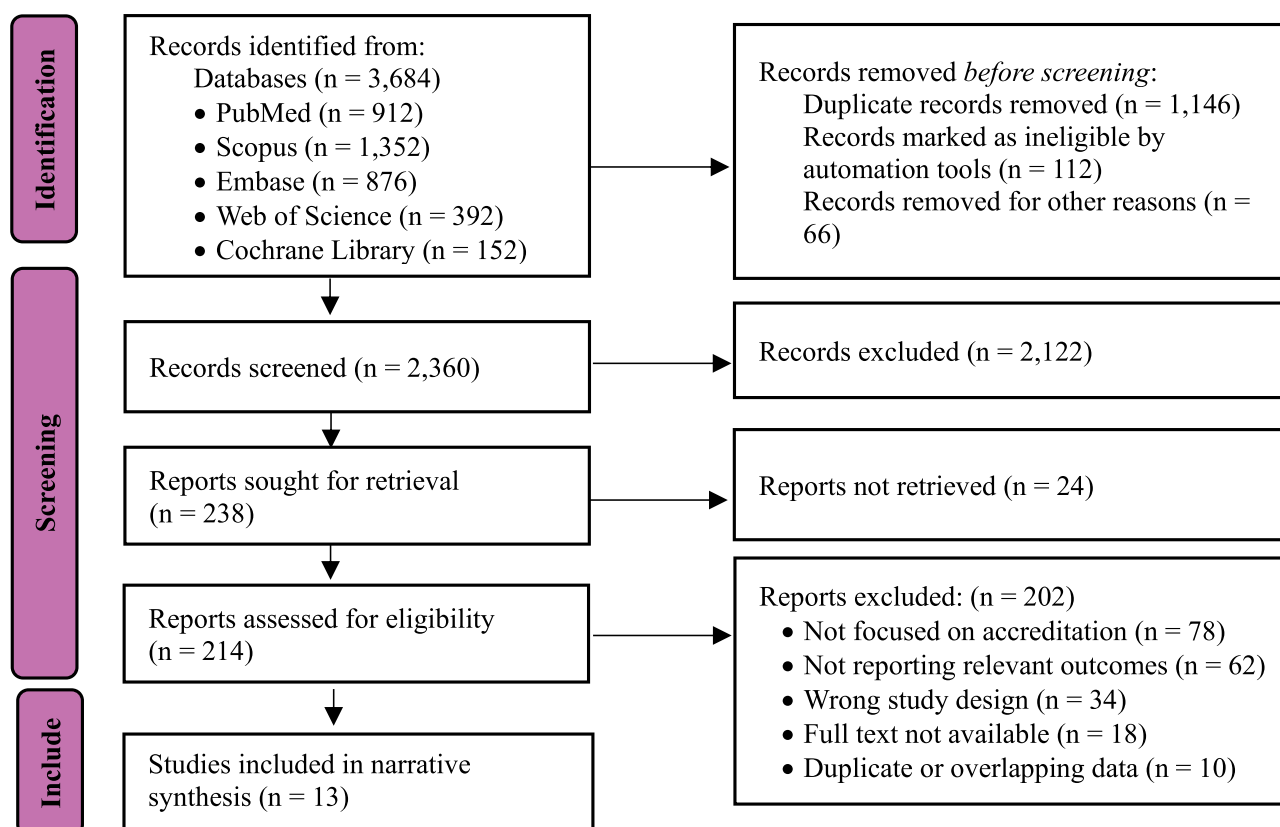


Fig. 1 PRISMA diagram of systematic review

Yousefinezhadi et al. 2020, Iran, $n=547$, mean=3.15, SD=0.66; El-Jardali et al. 2014, Lebanon, $n=403$, mean=4.27, SD=0.48). All three means lay above the neutral midpoint of 3, with one-sample Hedges' g of 1.10 [95% CI 0.93–1.26], 0.23 [0.14–0.31] and 2.64 [2.43–2.85] respectively. Heterogeneity between these studies was extreme ($I^2 = 99.6\%$), reflecting genuine cross-cultural variation in perceived effects rather than a single global effect; we therefore did not pool them and report each study individually.

Findings from qualitative, mixed-method, and quantitative studies were synthesized narratively following the SWiM reporting guidance; the three studies with comparable continuous outcomes are reported descriptively above.

Across studies, the magnitude of perceived improvement varied with health-system context, accreditation framework, and outcome scale used. Quality improvement seems greater in studies from high-income countries than in those from low- and middle-income countries, presumably because of the differences in resources and capacity for implementation.

Discussion

Accreditation evaluates health organizations' compliance with predefined performance standards to improve their standards and quality [24, 25]. A set of mandatory and voluntary approaches is conducted under the control of governmental and non-governmental bodies. The scope of accreditation can include a comprehensive health facility, specialty, or subspecialty [26, 27]. Important health-care organizations at the international level emphasize that attaining certification standards is an important tool to enhance organizational and clinical performance and recognize the role of accreditation as a valid indicator of quality [15, 28, 29]. However, the literature still lacks evidence to support this hypothesis. This review highlights the many positive experiences of hospitals in sustaining credit for hospital accreditation. The most important aspect is to enhance the quality of care and patient safety. Desveaux et al. and Babakkor and Kattan in their studies highlighted the potential of accreditation to foster a culture of safety, improve care coordination, and increase communication among healthcare teams [22, 30]. These changes are associated with an increased sense of responsibility for maintaining high standards and improving the morale of all staff members.

While this review draws on international data, the findings offer critical insights for the U.S quality field. The

Table 1 Summary of hospital accreditation challenges in various studies

S.No.	Title	Authors	Country	Study Setting	Study design	Sample size and population	Key findings	Research Gap
1	Accreditation and Quality Culture	Babakkor and Kattan (2023) [22]	Saudi Arabia	Tertiary Hospital	Cross-sectional	218 Healthcare workers	Accreditation positively associated with quality of care and organizational culture; group culture predominantly	Sample size; single center; quantitative approach
2	Managers' perspectives on accreditation	Yousefinezhadi et al. (2019)[23]	Iran	Hospitals	Cross-sectional	547 Mangers	Improved safety; dissatisfaction with process.	Limited stakeholder perspectives
3	Healthcare accreditation and organizational change	Carrasco-Peralta JA et al. (2018)	Spain	Healthcare Institutions	Qualitative	52 Healthcare professionals	Improved safety culture; increased bureaucracy	No clinical outcome assessment.
4	The impact of accreditation of primary healthcare centers	El-Jardali et al. (2014)	Lebanon	Primary Healthcare	Mixed method	25 Primary Health Centers	Improved documentation and Satisfaction	Weak financial support.
5	Accreditation implementation Challenges	Tashayoei et al. (2020)	Iran	Hospitals	Cross-sectional study	200 managers	Identified key Challenge dimensions.	Need outcome based standards.
6	Physician participation barriers	Ebrahimipour et al. (2021)	Iran	Hospitals	Qualitative	28 participants associated with accreditation process	Cultural and behavioral barriers	Need managerial support.
7	Accreditation and quality in healthcare: A grounded theory approach	Desveaux et al. (2017)	Canada	Healthcare organizations	Qualitative	239 Healthcare Organizations	Leadership influences sustainability	Limited generalizability
8	The perceptions of healthcare professionals on accreditation and its impact on quality	Katoue MG et al. (2021)	Kuwait	Hospitals	Qualitative	30 Healthcare Professionals	Improved safety; workload challenges	Need staff engagement
9	Impact of accreditation on the Workplace.	Alshamsi et al. (2020)	UAE	Hospitals	Qualitative	27 Healthcare professionals	Increased stress; Teamwork and coworker support	Limited policy focus
10	Ranking the Challenges of Accreditation	Zarifraftar et al. (2018)	Iran	Public & Private Hospitals	Descriptive research	280 participants	Identified key challenges.	Lack of Prioritization
11	European accreditation systems	Shaw et al. (2010)	Europe	Healthcare systems	Survey	183 organizations	Governance impacts sustainability	Limited sustainability focus.
12	staff experiences in Hospital accreditation processes	Al Mansour A, et al. (2022)	Saudi Arabia	Public Hospitals	Qualitative study	20 managers	Positive change; poor sustainability	Limited generalizability
13	Hospital Accreditation: as challenges / solution	Bastani P et al. (2021)	Iran	Hospitals	Qualitative	19 Participants	Structural and policy challenges	Need independent oversight

identified challenges, specifically administrative burden, “bureaucratization,” and staff stress – parallel the “compliance fatigue” often reported by U.S. clinicians during Joint Commission surveys. The finding that leadership support is the primary driver of sustainability reinforces U.S. models of “High Reliability Organizations.” Furthermore, the disparity in outcomes based on resource levels

suggest that U.S. safety-net hospitals may face similar sustainability hurdles as those seen in LMIC contexts, requiring tailored policy support to maintain quality gains.

Accreditation drives organizational improvement by increasing shared commitment to quality. Ebrahimi-pour et al. found that hospitals with good leadership and

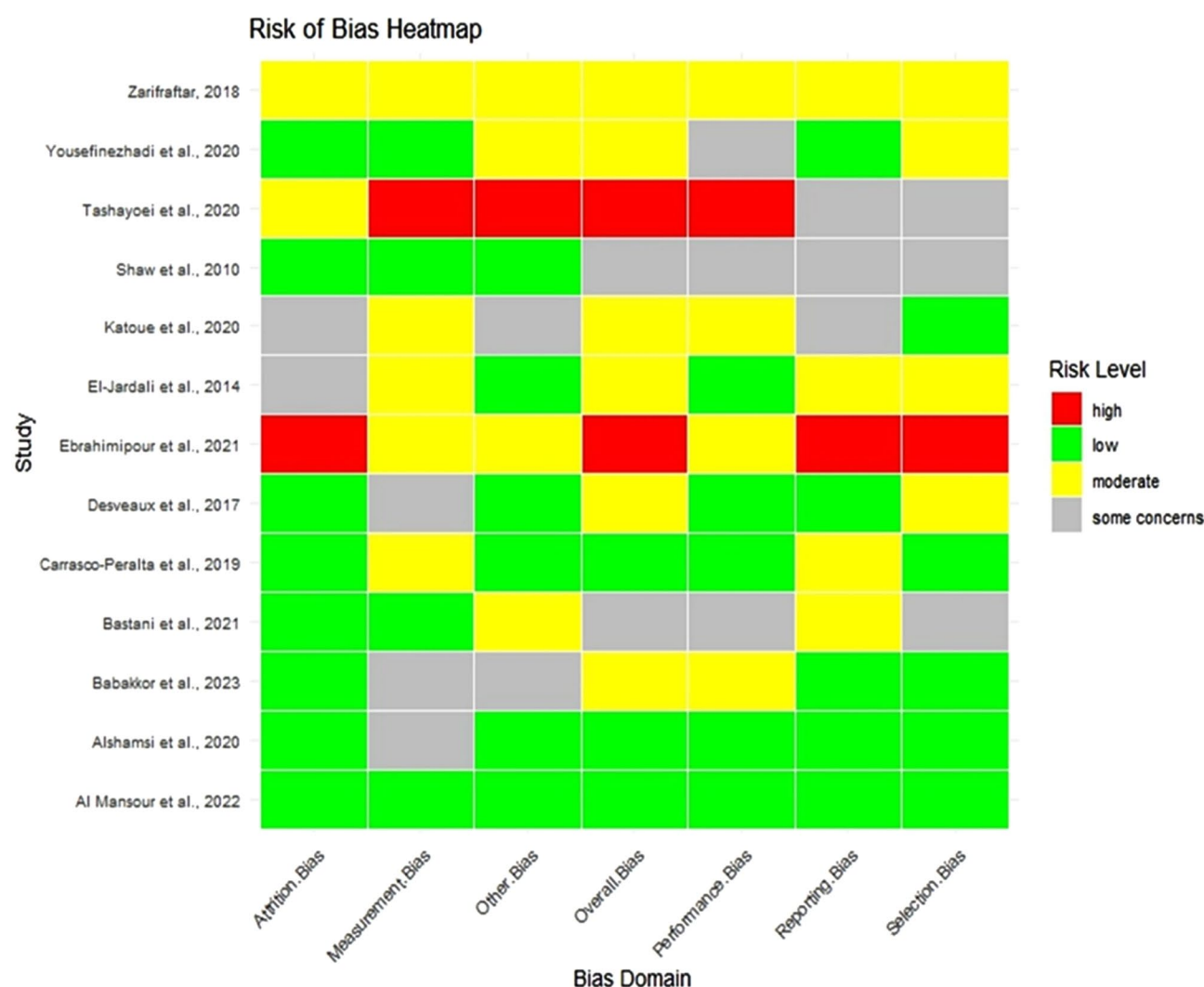


Fig. 2 Heatmap displaying the risk of bias across multiple studies, categorized by bias domains

active participation by staff undergo smoother transitions toward sustaining accreditation outcomes [31]. Hussein et al. and Alshamsi et al. highlighted persistent training and education as vital steps toward sustaining quality improvement [6, 32]. This supports the fact that through institutionalization of best practices and fostering professional growth, accreditation acts as a catalyst for long-term quality improvement. Notwithstanding positive outcomes, hospitals struggle to sustain their accreditation because of staff shortages, resource constraints, and financial limitations. Tashayoei et al. and Zarifraftar et al. noted that hospitals must tailor their accreditation requirements to their existing resources because understaffing and lack of funding may hamper their advancement [33, 34]. The burden for sustaining this will be even more evident when hospitals have to continuously document and monitor their performance without enough resources to effectively do so.

Ebrahimipour et al. found that hospitals with good leadership and active participation by staff undergo smoother transitions toward sustaining accreditation outcomes [31]. Building on this perspective, leadership commitment, robust governance structures, and continuous workforce development serve as enablers for embedding evidence-based practices into routine decision-making and sustaining accreditation gains through enhanced stakeholder engagement and a strong organizational culture [35]. Insufficient engagement and increased pressure at work make healthcare professionals resistant to accreditation. Coupled with the lack of leadership participation, this leads to poor outcomes. Ghazanfari et al. highlighted the structural adjustments to accreditation, especially in Iran, as mandatory and not necessarily compatible with the capacity or internal culture of hospitals [36]. According to Yousefinezhadi et al., institutions face obstacles to stakeholder participation [23]. If the key stakeholders involved are not the ones on

the frontline or the patients themselves, then the effectiveness of certification can be challenged. Communication and support at every level are required to maintain accreditation benefits, although variable and often limited participation is observed. Leadership commitment, governance structures, and workforce development emerged as a critical enabler for sustaining healthcare accreditation outcomes. Hospitals with strong leadership and active staff engagement demonstrate better integration of healthcare accreditation standards into routine practice, thereby enhancing long-term quality improvement and organizational culture.

Limitations

This study has the following limitations.

- The number of studies included in the meta-analysis was limited and precludes formal assessment of small-study effects, despite the I^2 result indicating considerable heterogeneity. Findings were therefore synthesised narratively following the SWiM reporting guidance (Campbell et al., *BMJ* 2020;368:16890; Supplementary File 3) rather than meta-analytically pooled.
- Substantial heterogeneity was observed due to variations in study design, settings and outcome measures.
- Several studies relied on self-reported data, which may introduce bias.
- Finally, differences between healthcare systems limit generalizability.

Implications

Policy and structural support are also important. El-Jardali et al. and Shaw et al. illustrated how the goals of accreditation must consider the financial plans and procedures of hospitals [29, 37]. Within supportive health systems that have financial and policy support, hospitals can sustain most of the benefits of accreditation. Tailoring the standards for accreditation to local contexts, as noted by Sahu et al., helps specialized areas such as mental health and improves quality [38].

For the most part, focusing on the efforts of hospitals and policymakers in a few key areas can help overcome the obstacles to accreditation. Strong leadership should be present in embedding accreditation into the culture of the organization for continuous improvement. Zarifrahtar et al. and Tashayoei et al. referred to accrediting standards that should be acceptable and attainable according to the specific needs and resources of hospitals in their study [33, 34]. This study brings together, for the first time, a structured narrative synthesis (following SWiM reporting guidance) of thirteen studies on the sustainability of healthcare accreditation,

complemented by descriptive reporting of three studies that provided continuous outcome data on accreditation impact. The narrative synthesis indicated that accreditation is consistently associated with positive perceptions of patient safety, organizational performance, and quality of care, with magnitude varying by health-system context. Additionally, this advantage tends to be greater in high-income countries than in low- and middle-income countries, indicating disparities in resources and implementation capabilities. This finding and a parallel qualitative synthesis of budgetary constraints and staff shortages provide a nuanced, evidence-based global perspective often absent in the literature [4]. Policymakers and healthcare accreditation bodies should design context-sensitive frameworks, ensuring financial, infrastructural, and workforce support, particularly in low-and middle-income settings.

Conclusions

The findings suggest that healthcare accreditation is associated with improvements in quality, patient safety, and organizational performance. However, the sustainability of these improvements varies across settings and is influenced by resource availability, governance, and implementation capacity. Strengthening contextual adaptation and leadership engagement is essential for sustaining accreditation outcomes. Differences in the healthcare system, accreditation processes, and available resources contribute to these varying outcomes. This is a valuable strategy for improving healthcare quality.

Supplementary Information

The online version contains supplementary material available at <https://doi.org/10.1186/s12913-026-14752-3>.

Supplementary Material 1: Supplementary File 1. PRISMA 2020 Checklist — The completed PRISMA 2020 Checklist for this systematic review is provided as a separate file accompanying this submission. Supplementary File 2. Detailed Search Strategy — Database-specific search strings and filters used during the literature search. Supplementary File 3. SWiM checklist — a detailed checklist as per Synthesis without meta-analysis in systematic review is provided as a separate file

Author contributions

All the authors contributed to the study, conception and design, performed title/abstract and full-text screening, contributed to the critical revision of the manuscript for intellectual content and approved the final draft submitted for publication. B. A. and G. M. conceptualized the review and drafted the first manuscript and narrative synthesis. R. M. and N. P. S. conducted a comprehensive review of relevant articles. A. A., A. K. and S. G. compiled and organized the review data into tabulated form and edited the results and discussion. G. M., A. A., B. A., and A. K. assessed the overall quality of the final manuscript.

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Data availability

No datasets were generated or analysed during the current study.

Declarations

Ethical approval

Not applicable.

Informed consent

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Competing interests

The authors declare no competing interests.

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